

Daily Content Intel

July 28, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, this one changed how I schedule speed work for my football and lacrosse athletes.

Researchers ran soccer players through 10 maximal 40 meter sprints, then retested them at 24, 48, and 72 hours. Posterior chain strength was still down at 48 hours in one leg and 72 hours in the other, and their pelvic control during the fastest part of the sprint kept drifting the wrong way across all 3 days.

The muscle is measurably weaker for 2 to 3 days after true top speed work, and it feels fine long before it actually is.

Keep sprinting. Running at 90 percent and above is the thing that builds a

hamstring that survives a game, and the kid who never touches top speed in practice is the one who grabs the back of his leg in the first quarter.

Just count it. Max velocity work goes early in the week, away from the game, and you don't stack a second speed day on top of a posterior chain that hasn't come back yet.

For my athletes that looks like speed on Monday, heavy lifting the same day, and by Thursday they're recovered enough to actually play Friday.

Count your true top speed reps. That's the dose.

Be Good. Help Someone. Learn Lots.

Hook: Your fastest sprint of the week leaves your hamstrings weaker for 3 days.

Spoken script (word for word):

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Thursday they're recovered enough to actually play Friday.

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On-screen text seeds:

- 0:00 10 max sprints. Hamstrings weak for 3 days.
- 0:12 Posterior chain strength still down at 48 and 72 hours
- 0:25 Pelvic control got WORSE as the days went on
- 0:38 Feels fine $\neq$ is fine
- 0:48 Sprinting builds the durable hamstring
- 1:00 Speed early in the week. Away from game day.
- 1:12 Count your true top speed reps

Caption (copy-paste ready):

Ten maximal 40 meter sprints. Then they retested the players at 24, 48, and 72 hours.

Posterior chain strength was still down at 48 hours in one leg and 72 hours in the other. Pelvic control during peak speed got worse across the 3 days, not better.

Keep sprinting anyway. Athletes who never touch 90 percent in practice are the ones who tear a hamstring in the first quarter of a real game. Top speed exposure is what makes the tissue durable.

Treat it as a dose. Max velocity work goes early in the training week, away from game day, and it stays off a posterior chain that's still recovering. With my football and lacrosse athletes that means speed and heavy lifting on Monday so they're fresh enough to play Friday.

Camp starts in a couple weeks. Plan the speed, count the reps above 90 percent.

Dr. Lars Stevenson, PT, DPT
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#hamstring #speedtraining #footballtraining
#lacrosse #strengthandconditioning
#sportsPT #athletedevelopment

#highschoolfootball #returntosport
#sprintmechanics

Personalize this

- Which of your HS football guys has come back from a hamstring strain, and how did you space their max-velocity days once they were sprinting again?
- Camp opens in a few weeks. What does your actual week look like for a skill player right now (speed day, lift day, conditioning) and where do you put the top-speed work relative to Friday?
- Have you had a coach schedule a hard speed session on a Thursday? What did you see on the field Friday, and how did you get that changed?

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC11556624/> | "Acute Changes in Hamstring Injury Risk Factors After a Session of High-Volume Maximal Sprinting Speed Efforts in Soccer Players"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC11556624/> | "Significant

small-to-moderate decreases in posterior chain muscles strength were detected until 48 hours"

- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC9414047/> | "Monitoring weekly sprint loading at velocities > 90% of maximum velocity may be valuable to help to reduce the risk of hamstring injuries in professional football."

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC11556624/>
- <https://pmc.ncbi.nlm.nih.gov/articles/PMC9414047/>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, a headline went around this week saying creatine spreads cancer, and I've already had parents of my athletes asking me about it. Let's look at what the paper actually did.

Most of it was mice. Researchers gave creatine, watched platelets turn hyperactive, and traced a signaling pathway through the bone marrow cells that make those platelets. That's real work, and the mechanism is genuinely interesting.

The human arm was 11 healthy adults taking 20 grams a day for 2 weeks. Cancer was never measured in those people. Their platelets got pulled out and injected into mice that had melanoma cells in them.

And 20 grams a day for 2 weeks sits way above a normal dose. That's a loading protocol, and you'd run it for 3 to 5 days before dropping down to 3 to 5 grams.

So here's my read, and I'm hedging where I should. If you're a healthy high school or college athlete, keep taking your creatine. It's probably the most studied supplement on the shelf and the safety record is long.

If you have active cancer or you're in treatment, that's a real conversation, and it belongs with your oncology team.

A mouse mechanism earns better human research. Keep the tub.

Be Good. Help Someone. Learn Lots.

Hook: "Creatine causes cancer" is trending. The human arm of that study had 11 people in it.

Spoken script (word for word):

Okay, a headline went around this week saying creatine spreads cancer, and I've already had parents of my athletes asking me about it. Let's look at what the paper actually did.

Most of it was mice. Researchers gave creatine, watched platelets turn hyperactive, and traced a signaling pathway through the bone marrow cells that make those platelets. That's real work, and the mechanism is genuinely interesting.

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And 20 grams a day for 2 weeks sits way above a normal dose. That's a loading protocol, and you'd run it for 3 to 5 days before dropping down to 3 to 5 grams.

So here's my read, and I'm hedging where I should. If you're a healthy high school or college athlete, keep taking your creatine. It's probably the most studied supplement on the shelf and the safety record is long.

If you have active cancer or you're in treatment, that's a real conversation, and it belongs with your oncology team.

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On-screen text seeds:

- 0:00 "Creatine causes cancer"
- 0:10 The human arm: 11 people
- 0:20 Mice. Platelets. A bone marrow pathway.
- 0:32 20 g/day for 14 days
- 0:42 Cancer was never measured in the humans
- 0:55 Normal dose: 3-5 g/day
- 1:08 Healthy athlete? Keep your creatine.
- 1:18 Active cancer? Ask your oncology team.

Caption (copy-paste ready):

A creatine and cancer headline made the rounds this week and I've already fielded it from parents.

Here's what the paper did. Most of it was mice. Creatine, hyperactive platelets, a signaling pathway traced back through the bone marrow cells that produce them. Mechanistically serious work.

The human arm was 11 healthy adults on 20 grams a day for 2 weeks. Cancer was never measured in any of them. Their platelets were removed and injected into mice carrying melanoma cells, and those mice metastasized more.

And 20 grams a day for 14 days is a loading dose that normally runs 3 to 5 days before dropping to 3 to 5 grams.

My read: a healthy high school or college athlete should keep taking creatine. The safety record is long and it's probably the most studied supplement on the shelf. Someone with active cancer or in treatment has a different risk calculation, and that question belongs in front of their oncology team.

A mouse mechanism earns better human research. Keep the tub.

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Be Good. Help Someone. Learn Lots.

#creatine #sportsnutrition #athletenutrition
#highschoolathlete #collegeathlete
#strengthandconditioning #sportsPT
#supplements #evidencebased
#footballtraining

Personalize this

- Which of your athletes (or their parents) actually asked you about this headline, and what did they say they were going to do about their creatine?
- What's your standard creatine guidance for a HS football or lacrosse athlete right now: dose, timing, and when you tell a parent to hold off?
- You've had post-op and oncology-adjacent patients in the clinic. How do you handle it when a patient brings you a scary supplement headline mid-plan of care, and how does that fit the CROSS framework?

Screenshots:

- title | <https://www.supplysidesj.com/healthy-living/experts-creatine-tumor-study-headline-exceeds-what-research-shows> | "Experts: Creatine/tumor study headline exceeds what research shows"
- physiology | <https://www.supplysidesj.com/healthy-living/experts-creatine-tumor-study-headline-exceeds-what-research-shows> | "The subjects took 20 g/d of creatine monohydrate for two weeks."
- actionable | <https://www.supplysidesj.com/healthy-living/experts-creatine-tumor-study-headline-exceeds-what-research-shows> | "And the 20 g/d dosage for two weeks is well beyond what is commonly recommended; a dosage that high typically is only used for three to five days before backing off to a maintenance dose of 3 g/d to 5 g/d."

Sources:

- <https://www.supplysidesj.com/healthy-living/experts-creatine-tumor-study-headline-exceeds-what-research-shows>
- <https://www.nature.com/articles/s41467-026-74639-z>

Reference

Runner-up topics

- Hulm et al., Scand J Med Sci Sports (June 2026): 79 NCAA D1 football players, MRI hamstring volumetry. Previously injured limbs kept muscle-specific structural deficits even after sprint kinematics looked normal. Perfect return-to-sport angle, but the article is paywalled and needs an accessible summary (or the author's preprint) before it can be cited with verbatim screenshots.
- In-season flywheel dose-response meta (BMC Sports Sci Med Rehabil, Jan 2026): 7 RCTs, 199 male athletes 16-23. Fewer than 2 sessions per week with about 12 total sessions produced the jump gains. Good in-season minimum-effective-dose Reel if reframed for gear a HS weight room actually owns.
- Iron Radio 245 on training age vs biological age and why teenage strength standards have exploded. Strong hot-take fuel about early specialization and what a

15-year-old should actually be chasing.

Sources scanned today

- **Newsletter digest**

(newsletter_digest_2026-07-28.pdf): 2 sources processed. Mike T. Nelson's Weekly Wrap Up (July 26) led with a myth-busting breakdown of the creatine/cancer headlines, plus Iron Radio ep. 245 on Colton Engelbreit's 2,700-lb total and why strength standards have exploded (training age vs biological age, early specialization risk). Dr. Mercola issue was pure supplement promo, skipped.

- **PubMed / PMC:** searched hamstring strain prevention, max-velocity sprint exposure, in-season resistance training frequency, youth speed development. Pulled Freitas-style acute max-speed dosing work (PMC11556624) and Shah 2022 weekly sprint-volume monitoring (PMC9414047), both verified in full.

- **Scandinavian J Med Sci Sports:** found Hulm et al. (June 2026) on prior hamstring injury, MRI muscle volume, and sprint

mechanics in 79 NCAA D1 football players. **Paywalled (HTTP 402), could not verify verbatim text**, so it moved to runner-ups rather than get cited unread.

- **MDPI Applied Sciences**
sprint-for-hamstring scoping review: HTTP 403, unreachable. Skipped.
- **Nature Communications** (Xie et al., creatine/metastasis, July 15 2026): article page auth-gated to WebFetch. Used as a secondary citation only; screenshot targets pulled from the accessible SupplySide expert breakdown.
- **BMC Sports Sci Med Rehabil**: in-season flywheel dose-response meta (Jan 2026) verified, held as runner-up (equipment too niche for a HS weight room).
- **Science for Sport / NSCA**: nothing new inside the 7-day window beyond evergreen youth-training position stands.